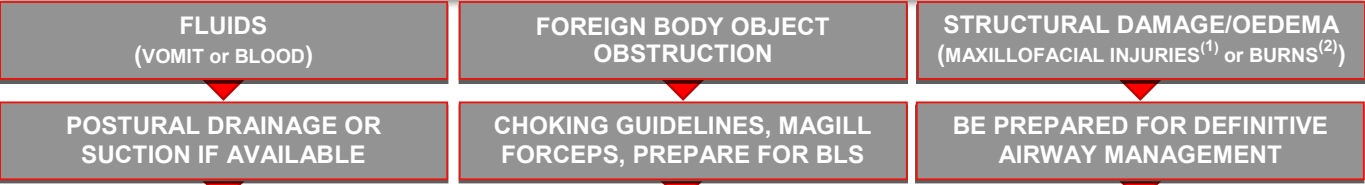


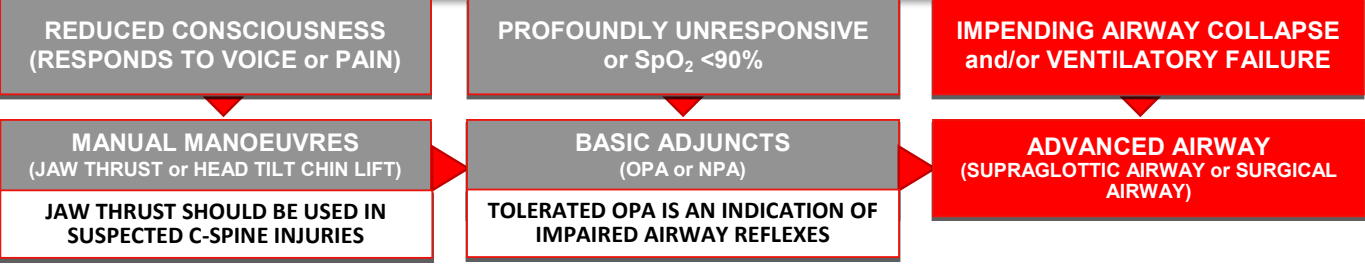
ADVANCED AIRWAY MANAGEMENT

AIRWAY LADDER APPROACH SHOULD BE USED. AIRWAY REFLEXES ARE CORRELATED WITH CONSCIOUS LEVELS. FOLLOW YOUR SCOPE OF PRACTICE WHEN USING GUIDE

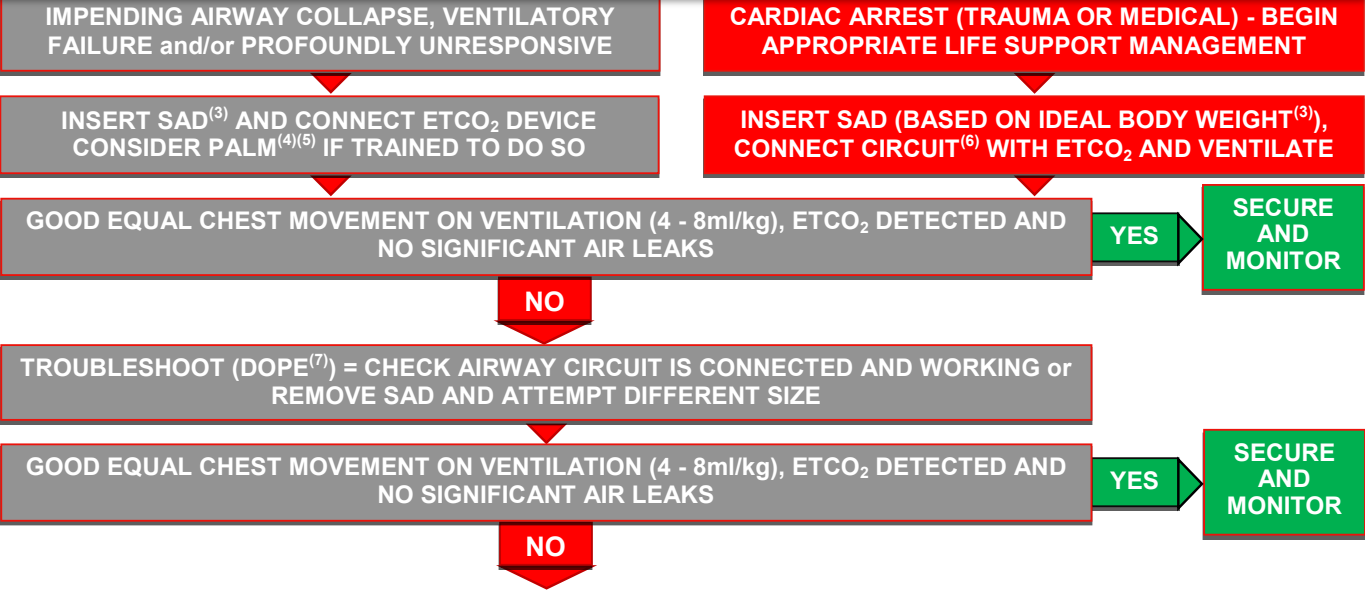
AIRWAY ASSESSMENT (CHECK AND CLEAR) BASED ON AVPU



AIRWAY MANAGEMENT (STEP WISE LADDER APPROACH)



SUPRAGLOTTIC AIRWAY DEVICE (SAD) INSERTION



CONSIDER IF MANAGEABLE BY BASIC ADJUNCTS AND BVM OR PERFORM SURGICAL AIRWAY

SURGICAL AIRWAY (DEFINITIVE) - SCALPEL, BOUGIE, TUBE TECHNIQUE

INDICATIONS	SEVERE UPPER AIRWAY BURNS (SIGNIFICANT SWELLING, STRIDOR AND DYSPNOEA) ALL OTHER AIRWAY TECHNIQUES HAVE FAILED (CAN'T INTUBATE CAN'T OXYGENATE PLAN) SIGNIFICANT MAXILLOFACIAL INJURY REQUIRING DEFINITIVE AIRWAY (REFER TO GUIDE) SEVERE LARYNGOSPASM CAUSED BY PALM TECHNIQUE
EQUIPMENT NEEDED: SCALPEL, SIZE 6mm ENDOTRACHEAL TUBE (ETT), INTUBATION BOUGIE	
1. MAINTAIN POSTURAL DRAINAGE (IF ABLE) WHILST PREPARING KIT DUMP. 2. PALPATE LARYNX, LOCATE AND HOLD CRICOTHYROID MEMBRANE. 3. MAKE A TRANVERSE STAB INCISION INTO MEMBRANE AND ROTATE 90°, KEEP BLADE IN SITU. 4. INSERT BOUGIE AND ADVANCE 10cm (MAY FEEL CARTILAGE RINGS), REMOVE BLADE. 5. PASS ETT OVER BOUGIE AND ADVANCED 8 - 10cm INTO TRACHEA THEN REMOVE BOUGIE. 6. INFLATE CUFF AND SECURE ETT WITH TUBE TIE, NOTING DEPTH OF TUBE. 7. CHECK VENTILATION AND EtCO₂ – NO EtCO₂ = NO PLACEMENT = REATTEMPT	

SUCTION ASSISTED LARYNGOSCOPY AIRWAY DECONTAMINATION (SALAD)

INDICATION: SIGNIFICANT CONTINUOUS AIRWAY CONTAMINATION REQUIRING EMERGENCY SUCTIONING TO FACILITATE LARYNGOSCOPY.

PERFORM:

1. PREPARE THE SUCTION UNIT & LARYNGOSCOPE. MAINTAIN POSTURAL DRAINAGE.
2. SLOWLY INSERT THE SUCTION CATHETER ALONG THE TONGUE UNTIL UVULA IS SEEN, INSERT SCOPE TO LIFT THE TONGUE UNTIL THE EPIGLOTTIS IS SEEN.
3. ADVANCE SUCTION CATHETER INTO HYPOPHARYNX UNTIL CORDS ARE VISIBLE.
4. PLACE SUCTION CATHETER TO LEFT OF LARYNGOSCOPE, KEEP IN HYPOPHARYNX.
5. KEEP SUCTIONING UNTIL AIRWAY IS SECURE WITH DEFINITIVE AIRWAY.

POINTS TO NOTE

1. FOLLOW MAXILLOFACIAL INJURY GUIDELINE. KEEP PATIENT IN POSITION THAT MAINTAINS THE AIRWAY.
2. HAVE A LOW THRESHOLD FOR REQUESTING SUPPORT FROM A LEVEL 8 CLINICIAN THAT CAN PERFORM PREHOSPITAL EMERGENCY ANAESTHESIA (PHEA) FOR A DEFINITIVE AIRWAY. PATIENTS WITH SIGNIFICANT AIRWAY BURNS, SWELLING AND STRIDOR REQUIRE IMMEDIATE DEFINITIVE AIRWAY (SURGICAL AIRWAY).
3. SAD's ARE SIZED USING IDEAL BODY WEIGHT BUT CONSIDER INDIVIDUAL ANATOMICAL VARIATIONS (SUCH AS WIDENED THYROID CARTILAGE MAY REQUIRE A LARGER SIZE THAN SUGGESTED, WHEREAS OBESE PATIENTS MAY REQUIRE A SMALLER AIRWAY THAN THEIR WEIGHT SUGGESTS).
4. PHARMACOLOGICALLY ASSISTED LARYNGEAL MASK (PALM) INSERTION MAY BE CONSIDERED FOR THE UNRESPONSIVE PATIENTS WHERE ADVANCED AIRWAY MANAGEMENT IS INDICATED BUT HAVE PERSISTING AIRWAY REFLEXES THAT PRECLUDE SUPRAGLOTTIC AIRWAY INSERTION. A SEDATION DOSE OF KETAMINE (1mg/kg) MAY BE USED IN THESE CIRCUMSTANCES. CONTINUOUS ET_{CO}₂ MONITORING IS MANDATORY IN THESE PATIENTS AFTER AIRWAY INSERTION.
5. KETAMINE CAN PRESERVE AIRWAY REFLEXES, LIMITING THE VIABILITY OF THIS TECHNIQUE. MOREOVER, PALM CAN CAUSE LARYNGOSPASM, WHERE A SURGICAL AIRWAY WOULD BE REQUIRED. PALM SHOULD ONLY BE PERFORMED BY TRAINED PRACTITIONERS AND SHOULD NOT DELAY PRIORITY OF GETTING A LEVEL 8 CLINICIAN TO THE PATIENT FOR PHEA AND DEFINITIVE AIRWAY.
6. AIRWAY CIRCUIT COMPRISES OF CATHETER MOUNT, HME FILTER, ET_{CO}₂ CAPNOGRAPHY THEN BVM OR VENTILATOR.
7. DOPE MNEUMONIC USED FOR TROUBLESHOOTING INEFFECTIVE ADVANCED AIRWAY MANAGEMENT.
D - DISPLACEMENT OF AIRWAY DEVICE, O - OBSTRUCTION (SECRETIONS/BLOOD PLUG or KINK IN CIRCUIT),
P - PNEUMOTHORAX, E - EQUIPMENT (EVERYTHING HOOKED UP CORRECTLY AND WORKING?).
8. SIGNS OF INEFFECTIVE SUPRAGLOTTIC AIRWAY INSERTION INCLUDE SIGNIFICANT AIR LEAKING, INABILITY TO SEE TRACE OF ET_{CO}₂, NO CHEST MOVEMENT ON VENTILATION AND NO SOUNDS ON AUSCULTATION.

Guideline written in accordance with SFM Clinical Standard Operating Procedure 002 'Advanced Airway Management' (2022).

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